



Night sweats have many possible causes, from minor through to life-threatening cancers or infections. We cannot list them all here or tell you what investigations to do for each possible cause. Instead, we have highlighted the common and serious causes to provide a framework within which to consider the patient's symptoms and our choice of investigations. Our aim is to **RULE IN** likely possibilities and to **RULE OUT** the more serious causes.

Assessment of night sweats

- **Think about the character and context of the sweats:** duration, frequency and severity.
 - *Recent onset of severe sweats is more concerning than intermittent mild sweats over a longer timeframe.*
- **Take a detailed history (and appropriate focused examination), identifying other symptoms/signs that are present and looking for red flags.**
 - *High-risk signs or symptoms warrant immediate work-up or referral.*
 - *Lower-risk cases may be observed for 2–4 weeks before considering whether tests are needed.*
- **Consider risk factors,** e.g. travel, contact with infectious diseases.

Red flags

Night sweats with any of these features warrant urgent investigation or referral according to your local pathways:

- Fever
- Lymphadenopathy
- Weight loss
- Unexplained bruising, bleeding or rash
- New heart murmur
- Cancer in past medical or family history
- History of TB/latent TB
- Active IV drug user
- High-risk sexual history
- Immune compromise

No red flags

- Choose initial investigations according to the character and context of the night sweats.
- In the absence of red flags, watchful waiting may be appropriate for 2–4 weeks before investigating.

Causes of night sweats

4 major groups to consider (this is not an exhaustive list, but helps us consider more common/serious causes).

INFLAMMATORY

Malignancy

- Leukaemia, lymphoma
- Solid organ tumours

Infection

- TB
- HIV
- Bacterial endocarditis
- EBV and other respiratory viruses
- Lyme disease
- Tropical infections

Autoimmune

- Sarcoidosis
- Rheumatoid arthritis
- Giant cell arteritis

DRUGS

SSRIs
Beta-blockers
Anticholinergics
Androgen depleters (e.g. aromatase inhibitors, tamoxifen, GnRH analogues)

ENDOCRINE

Hyperthyroidism
Menopause
Hypogonadism
Pheochromocytoma
Hypoglycaemia

OTHER CAUSES

Sleeping environment
Gastroesophageal reflux
Obstructive sleep apnoea
Substance use, including alcohol

We make every effort to ensure the information in these pages is accurate and correct at the date of publication, but it is of necessity of a brief and general nature. The information presented herein should not replace your own good clinical judgement, or be regarded as a substitute for taking professional advice in appropriate circumstances. In particular, we suggest you carefully consider the specific facts, circumstances and medical history of any patient, and recommendations of the relevant regulatory authorities. We also suggest that you check drug doses, potential side-effects and interactions with the British National Formulary. Save insofar as any such liability cannot be excluded at law, we do not accept any liability for loss of any type caused by reliance on the information in these pages. May 2024. For full references see the relevant Red Whale articles.